ClinicalKey AI

Frequently Asked Questions

- Data Privacy and Security
- User Interface
- Content
- Technology
- Safety and Fairness
- Deep Research



Question	Answer
Data Privacy and Security	
Are you HIPAA compliant?	Yes, ClinicalKey AI is designed to meet HIPAA requirements for US customers. We maintain end-to-end platform control and implement controls to safeguard Protected Health Information (PHI) and Personally Identifiable Information (PII). • **US customers:** A Business Associate Agreement (BAA) is available. If your organization has an active BAA in place, limited (i.e., minimally necessary) PHI or PII may be included in the query field; check with your administrator if unsure. • **Non-US customers:** HIPAA and BAA provisions do not apply. Inclusion of PHI or PI in the query field is not permitted.
User Interface	
What are recent changes to the user experience?	There are several new changes to the user experience for February 2026. The most visible will be the all-new home page that allows for custom views and more control over previous queries (including saving and removing favorites), the ability to toggle between adult and pediatric specifications in query responses and query assistance to help users ask clearer clinical questions.
Are non-English languages now supported?	Yes, users can ask questions in languages other than English. The system translates the question into English for retrieval, generates the answer, and returns it in both English and the original language (users can toggle between languages using the “See in” button); the underlying content is US-English, and while non-English inputs may be supported, non-English responses have not been validated.

Question	Answer
Content	
What content sources does ClinicalKey AI use?	ClinicalKey AI relies on high quality clinical content including journal article abstracts, medical textbooks, full text clinical overviews and drug monographs and Government publications from the FDA, CDC and NLM. For more information on content sources, please visit this page.
How often does ClinicalKey AI update its content sources?	Update frequency varies by content type. Elsevier enables a pipeline to supply the ClinicalKey AI library with any new or updated content on a daily basis. Books are updated with any new editions or corrections. Drug reference and point of care content is updated as new topics are published or practice recommendations change.
What content changes have you made to support answers?	We retrieve from a more extensive content base, as of February 2026. We have doubled the volume of full-text journal articles, tripled the number of guideline sets, and added abstracts from the past 10 years from more than 1,100 journals. With this added content, the system is optimized to deliver more comprehensive responses that better support the user's next action.
What specialty areas does ClinicalKey AI cover?	We do not position ClinicalKey AI as focused on a single "top specialty"; instead, it is designed to support a broad, balanced range of clinical questions across specialties.
Will references be selected according to new ranking criteria? If so, which ones?	References are selected primarily for relevance. When available, the system prioritizes evidence types that commonly form the core of clinical decision-making—such as practice guidelines, meta-analyses, Cochrane reviews, governmental or expert panels, and large randomized controlled trials.

Question	Answer
Content (continued)	
What's a journal impact factor? How do you determine the sequence or rank of the content types in the response?	Journal Impact Factor is a widely used journal-level metric that reflects how frequently, on average, articles in a journal were cited over a two-year period. Higher values generally indicate that a journal's articles are cited more often. We use a combination of Impact Factor and FWCI (Field- Weighted Citation Impact) when selecting journal content. FWCI measures how often a publication is cited compared to the global average for similar publications, normalized by discipline, publication type, and year.
What is a link resolver? Does my institution need any specific set up to use it?	A link resolver connects abstracts in ClinicalKey AI to your institution's collections so users can access the associated full-text articles. This helps users reach non-Elsevier content available through your institution, in addition to the full-text content accessible via ClinicalKey. Our implementation specialists can provide the documentation and instructions needed to set this up quickly and efficiently.

Question	Answer
Technology	
Is ClinicalKey AI available via IP Auth verification?	ClinicalKey AI requires an individual user login to use its generative AI capabilities and personalized features.
Which large language models (LLMs) does ClinicalKey use?	ClinicalKey AI uses a combination of commercial and proprietary large language models (LLMs), plus additional components in our retrieval-augmented generation (RAG) workflow. Because foundation models evolve quickly, we continuously evaluate and update models based on performance for our use case—there is not a single, fixed LLM. Within the workflow, we interpret and expand the user's query to support broad and deep retrieval across a diverse content library using multiple retrieval methods. We then validate the response against the cited sources in real time and adjust wording to ensure claims are accurate and appropriately contextualized.
How does ClinicalKey AI manage hallucinations?	ClinicalKey AI reduces hallucinations via its "Open book exam" design where every question is answered by finding relevant content from a carefully curated library of clinical information. By comparison, LLMs work like a "Closed book exam" where answers are generated based on patterns learned from its training data without relevant content to ground it.
How does ClinicalKey AI respond to questions with limited or emerging research?	ClinicalKey AI answers reflect the underlying evidence. When there is little or conflicting evidence, the response will reflect that in its response and moderate its own certainty in accordance to the degree of evidence and consensus underlying the answer. If there is insufficient information to answer the question at all, the response will also indicate that.
Will AI responses change?	Responses follow the same overall structure: a direct answer at the top, followed by supporting bullet points that add context. We have added bold lead-in phrases to bullet points for faster scanning. Key improvements include more comprehensive and detailed answers, reduced anchoring/confirmation bias from the initial query, and clearer surfacing of safety-relevant information.

Question	Answer
Safety and Fairness*	
How does ClinicalKey AI address health equity?	Elsevier is firmly committed to promoting health equity and reducing bias in healthcare. With ClinicalKey AI, Elsevier includes clinical overviews and drug monographs that include information on special populations, and users can include specific characteristics like race/ethnicity in their queries. Elsevier also has a team of clinicians who evaluate responses for bias on an ongoing basis.
How does ClinicalKey AI address product safety?	Elsevier is highly committed to product safety and has put many measures in place to help deliver quality. This includes curating the training data for proprietary models, designing a system that grounds responses in content, and daily refreshes of that content. Elsevier also has a network of hundreds of subject matter experts we have tapped to regularly evaluate the answers we generate. In addition, we give users the opportunity to flag questionable answers that get reviewed.
How does Elsevier ensure ClinicalKey AI generates trustworthy answers?	Elsevier helps to ensures quality answers by basing responses on relevant content from a carefully curated content set. Answers include citations and snippets of the content used to answer the questions for easy fact checking. Elsevier also has a robust, ongoing evaluation process that includes external clinician subject matter experts.
How do you ensure answer quality and correctness?	We conduct subject-matter expert evaluations to monitor response quality, and we run additional ad hoc evaluations for significant releases. Our evaluation methodology is described in our published study and is continually improved. We have also increased the number of citations per response. Users will now typically see 6–10 evidence-based, linked citations in every ClinicalKey AI response.

**To learn more about Elsevier's Responsible AI Guidelines, please refer to this page.*

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Question	Answer
Deep Research: General Overview	
What is CKAI Deep Research?	CKAI Deep Research is an AI-powered research capability inside ClinicalKey AI (CKAI) that extends the product from fast, point-of-care answers into structured, citation-rich, topic-level evidence synthesis. It takes a broad research question and returns a long-form report (roughly 3,000-5,000 words) citing 20+ peer-reviewed sources, built for learning, teaching, evidence review, and protocol/committee discussion.
What is the simplest way to describe it?	Standard CKAI is optimized for fast, focused answers. Deep Research is optimized for broader, deeper evidence review across multiple sources. Think of Deep Research as a high-quality evidence briefing, not a verdict, guideline, or systematic review.
What is Deep Research NOT?	• Not a systematic review or meta-analysis • Not a clinical guideline • Not a point-of-care decision support tool • Not a patient-specific recommendation engine
What problem does Deep Research solve?	ClinicalKey AI is strong at rapid, evidence-grounded answers, but users who need deeper topic exploration often leave the product entirely - turning to ChatGPT/Claude, PubMed, or manual literature review. This creates three problems: (1) workflow fragmentation - users leave the product to research elsewhere; (2) time burden - deeper research can take hours of searching and organizing sources; and (3) a trust/reuse gap - generic AI tools are convenient but lack transparent sourcing and reusable, evidence-oriented structure. Deep Research keeps that deeper-research workflow inside the CKAI ecosystem.

Question	Answer
Deep Research: Content & Data Sources	
What content does Deep Research use?	Deep Research draws primarily on the Scopus abstract corpus (~90M+ documents from 7,000+ publishers) via a Shared Search API and search. This is a major departure from standard CKAI.
Can Deep Research search the open web?	No. Results depend entirely on scholarly content that is indexed, available, and relevant within the Scopus-based corpus - it does not perform open web search.
Does it use local/regional guidelines (e.g., NICE)?	No.

Question	Answer

Deep Research: Content & Data Sources

What is the difference between “ClinicalKey AI Point of Care" and "ClinicalKey AI Deep Research"?

Dimension	CKAI POC	CKAI Deep Research
Primary data source	ClinicalKey full-text content	Scopus abstract corpus
Corpus scale	~3.27M documents / ~28.9M chunks	~90M+ documents, 7,000+ publishers
Content depth	Full-text (entire book chapters, journal articles, clinical overviews, drug monographs)	Abstracts only (title, abstract, metadata, citations)
Content type	Curated clinical content: 900+ journals, 40+ books, 100+ society guidelines, clinical overviews, drug info, procedures	Peer-reviewed journal articles, conference papers, book items, preprints — broad academic/biomedical literature
Clinical curation	Clinically curated and validated — clinician-in-the-loop process	Scopus bibliographic database.
Output style	Concise, clinically focused answer for point-of-care use	Long-form report (~3,000–5,000 words) with evidence, caveats, traceable sources
Report structure	Lightweight: direct answer, key points, optional short summary	Structured: executive summary, methodology, findings, evidence tables, limitations, references
Persona/framing	POC assistant for rapid experimentation, usability testing, workflow validation	Expert research analyst / evidence reviewer for deeper, source-backed investigation
Generation time	Minutes or less, depending on query complexity	Minutes (typically ~3 min, up to 10–15 min for complex queries)
Depth of analysis	Optimized for speed and iteration based on use case of Point of care.	High depth, broader search coverage, synthesis, critical interpretation
Citations	Limited sources	Explicit citations (15–20+ sources), source confidence, reference package
Best suited for	Point of care	Topic deep dive, research tasks, comparative analysis, evidence review, strategic decision support

<table>
<tr><th>Question</th><th>Answer</th></tr>
<tr><td colspan="2">Deep Research: Safety, Scope & Appropriate Use</td></tr>
<tr><td>Can Deep Research be used for patient-specific decisions?</td><td>No. It is explicitly not intended for patient-specific diagnosis, treatment, dosing, staging, scoring, or point-of-care decision-making. It is designed for topic-level evidence review, learning, and discussion support. The product is not intended to replace clinical judgement or guide individual patient care.</td></tr>
<tr><td>What happens if a user asks a patient-specific question?</td><td>Deep Research includes patient-safety guardrails that detect patient-specific intent and redirect the user with reframing guidance, helping them convert the question into a safe, topic-level research question.</td></tr>
<tr><td>Can you give examples of reframing a patient-specific question?</td><td><table>
<tr><th>Not appropriate (patient-specific)</th><th>Reframed as a safe research topic</th></tr>
<tr><td>What should I do for my 77-year-old patient with AF and fall risk?</td><td>What does the evidence show about anticoagulation risks and benefits in older adults with atrial fibrillation and elevated fall risk?</td></tr>
<tr><td>This patient has a penicillin allergy - what antibiotic for endocarditis?</td><td>Summarize the evidence for alternative antibiotic regimens in infective endocarditis for patients with beta-lactam allergy.</td></tr>
</table></td></tr>
<tr><td>Can users trust the output of a Deep Research report?</td><td>Reports include citations, but users should check important claims against the cited sources and apply professional judgment. Deep Research is a high-quality briefing and synthesis tool - not a validated systematic review or guideline - so it should be treated as a starting point for deeper investigation, not a final authority.</td></tr>
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Question	Answer
Deep Research: Safety, Scope & Appropriate Use	
Is patient data (PHI/PII) sent to the underlying AI models?	Query text can optionally be de-identified before processing. When de-identification is enabled, PHI/PII is stripped from the query before it is sent to guardrails or upstream to the Shared GenAI backend. If de-identification fails for any reason, the system 'fails closed' - it raises an error rather than risk sending an unprocessed query upstream.
Is customer data used to train the underlying AI models?	No.
Deep Research: Prompt Length, Character Limits & Input Validation	
What is the current character limit for Deep Research queries?	Deep Research queries currently have a hard limit of 500 characters for the initial release.
Is there a character count indicator or limit notification in the UI?	There is currently no visual counter or warning message displayed. Once the input reaches 500 characters, the field stops accepting additional text.